Ysbyty Gwynedd
Anaesthetics Department
Adran Anaestheteg

OPERATIONAL

Out of Hours Inter-Hospital Transfers by Ysbyty Gwynedd Critical Care Teams

YG OPG 017

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Key Points

- The transfer of critically ill patients is potentially hazardous especially if poorly executed.
- Only Time-Critical Transfers are to be undertaken out of hours.

Introduction

Approximately 40 patients per year require an Inter-Hospital transfers from Ysbyty Gwynedd outside of normal working hours. This guide aims to provide an overview to ensure we continue to deliver high quality patient transfer care.

Which Patients?

The decision to transfer and to accept a patient must be made by appropriate consultants in both the referring and receiving hospitals. Only those patients in need of time critical interventions **not** provided locally should be transferred over night. Patients who are requiring organ support, invasive blood pressure monitoring and/or are at high risk of airway complications, require anaesthetic/critical care team escort.

Capacity transfers should only be undertaken out of hours in exceptional circumstances. In view of this, when there is no local capacity for a patient requiring admission to Critical Care, joint decision making between the 1st On Consultant, ICU shift leader, ODP & Recovery Nurses is essential in determining the best location & staff to manage the patient overnight.

Suggested options:

1. Manage on ICU/HDU if bed space available, but no nursing staff available utilising Recovery Nurse to assist.
2. Relocate an existing low acuity ICU/HDU patient to Recovery to enable admission of new patient to Critical care.

If lack of capacity is unlikely to be resolved in the morning, then plan for a daytime Capacity Transfer ideally utilising ACCTS. Should the 1st On Call Consultant deem a capacity transfer the only option to ensure patient safety, it is important to discuss with the hospital Bronze on call as well as the patients' family to facilitate shared decision-making.

Which Team?

It is the responsibility of the 1st On Call Consultant to determine which Doctor is the most appropriate to undertake the transfer.

The transfer team is to comprise of an ODP or critical care nurse plus an anaesthetist with the skills of CT3/ST3 or above.

It is expected that both team members should have the following:

- Adult & Paediatric Resuscitation certification
- Completed Part A & Part B Transfer Course

Conveyance

Firstly, contact the EMRTS Critical Care Hub (**0300 123 2301**) to discuss consideration for transfer. If they are unable to accept, document the reasons in the notes.

If EMRTS is unavailable then request a WAST asset as early as possible, stating to the dispatcher that the patient requires an 'IMMEDIATE transfer for LIFE SAVING TREATMENT'.

Patient Preparation

The transfer of critically ill patients is potentially hazardous especially if poorly executed.

Appropriate resuscitation and stabilisation of the patient before transfer is key to reducing the physiological disturbance associated with movement and mitigating the risk of deterioration during the transfer.

The Transfer Checklist must be used before departing (Appendix 1) to help ensure that all necessary preparations have been completed.

Where possible, patients & relatives should be informed of the indication and risks of Transfer and the conversation documented in the notes.

Equipment

Our hospital is equipped with dedicated Adult and Paediatric transfer trolleys and kit bags.

Location:

Adult Trolley - Cybi store room
Adult Kit Bag - Cybi store room
Paediatric Trolley – Minfford store room (Door: C12345)
Paediatric Kit bag – Theatres store room
Transfer Monitor – one Attached to Adult Trolley & one in Cybi, by the Resus trolley
Hamilton T1s – Attached to Adult Trolley & more in Ventilator store room, Cybi

Checking Responsibilities:

When re-stocking and checking kit, the designated checklist must be used and kit bags retagged.

- Transfer Team: responsible for ensuring that equipment is cleaned & re-stocked on return. (N.B. If they arrive back after the end of their shift, this can be delegated to the day on call ODP.)
- ITU housekeeper: checks transfer ventilator daily and ensure it is plugged in.
- ITU housekeeper: checks glidescope on alternate days.
- ITU RN: checks transfer bag once a week.
- On call ODP: checks bag is tagged each night.

On Return to Base

Transfer kit should be cleaned and restocked, as above.

The Doctor should have a minimum of 11 hours rest between leaving work and starting their next shift. If this impacts on a subsequent shift, agree a start time with the 1st On Consultant. The 1st On Consultant needs to arrange additional cover for this gap. Delegate to the rota team if in week day hours.

Place a copy of the Transfer document in the audit box (see below).

Documentation

Use the All Wales Critical Care Network Transfer document. These are located in the Doctor's office, Cybi. There are three copies: one for the receiving hospital, one for the patient & one for audit (see below). The last two are carbon copies so it is important that you write legibly and firmly as well as completing all boxes.

Recording vital signs can be challenging, but should be done every 5 min, as per AAGBI standards. Document any adverse events or difficulties & actions taken.

Training

Transfer staff will be expected to complete and be provided with the following local training:

Knowledge Based:

- Part A is an online knowledge based course. Link: Course: 000 NHS Wales - Transfer Course (esr.nhs.uk)
- Trauma Team Cymru Manual (Ch.23&24) Link: Tim Trawma Cymru Manual 2023 | Adran Anaestheteg Ysbyty Gwynedd (anaesthesiaforum.com)

Practical Based:

- Part B is an in-person, ½ day course covering equipment, packaging, ambulance orientation & simulation.

Patient & Staff Safety & Support

Warm clothing, ideally high-visibility jackets, should be worn by the team Theatre shrubs and/or 'crocs' are not appropriate. A mobile phone and money for food and drink on the return journey is useful, particularly if there is a delay. Many clinicians suffer from motion sickness and this should be addressed before leaving.

In the ambulance, staff, patient and equipment must be restrained. Ensure the patient is restrained including shoulder straps.

The Transfer Team should have the 1st On Call Consultant's contact number(s) for support & advice. Inform the Consultant of any adverse event occurring en route.

All members of staff involved in transporting patients should ensure that adequate financial arrangements are in place for themselves and their dependents in the event of an accident. The Anaesthesia Department procures personal indemnity insurance cover annually, but individuals may wish to obtain additional cover through membership of the ICS or AAGBI.

Audit

A copy of the Transfer document should be returned and placed in the marked collection box, located in the Doctor's office, Cybi. All Transfer documentation will be reviewed for quality & completeness.

Adverse events and/or challenging cases should be highlighted to the M&M Lead for presentation & discussion at the next meeting or CG day. A Datix submission is required for any incidents. Include 'Critical Care Transfer' as an identifier to enable future searching & analysis.

Recommended Additional Reading

- AAGBI Inter-hospital Transfer 2009
- AAGBI Updated Guidelines on the Safe Transfer of Patients with Brain Injury 2019

References

- The Transfer Of the Critically Ill Adult, ICS Guideline 2021
- Transfer of the critically ill patients, BJA Education, 18(3): 63e68 (2018)

Appendix 1: Transfer Checklist

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Pre-transfer Checklist. (For transfers to another hospital)

	1. VENTILATED PATIENTS *(non ventilated → proceed to section 2)*
☐	ETT position checked & secured.
☐	Capnography trace checked. End tidal CO2 Target?
☐	Ventilator Settings checked?
☐	PaO2 & PaCO2 on blood gas optimal?
☐	Thoracostomies or Chest Drains required?
☐	Oesophageal temperature probe in situ? Target temperature?
☐	Pupil response checked. Eyes protected ?
☐	Neuroprotection elements optimised? (if applicable) (**see reverse**)
	2. ALL PATIENTS
☐	Patient ID wristbands x 2 and correct?
☐	Acceptance by receiving unit specialty and ward confirmed?
☐	Transfer team appropriately dressed.
☐	Patient warming as appropriate.
☐	Patient notes & referral letter?
☐	Patient belongings packed?
☐	Transfer documentation started?
☐	IV access x2– flushed, secured & accessible with 3 way tap & extension?
☐	Snag risk minimised (lines and fluids).
☐	NIBP in situ on regular cycles.
☐	Arterial line and transducer accessible?
☐	Monitor/syringe drivers/ventilator secured?
☐	Tube & line position checked in CXR/CT?
☐	Urinary Catheter checked and bag emptied?
☐	Anti-emetic given: patient & team?
	3. EQUIPMENT TO HAVE IMMEDIATELY ACCESSIBLE
☐	Transfer bag present?
☐	Intubation pack including BVM + LMA or iGel.
☐	Sedation?
☐	Muscle relaxant?
☐	Vasopressors *(Infusion plus bolus)?*
	4. EQUIPMENT & DRUGS
☐	Monitoring attached: ECG? SaO2? BP Cuff? Capnography? A-line?
☐	Batteries checked on all equipment?
☐	Sufficient oxygen?
☐	Blood box (if applicable) plus Transfusion document present?
☐	Other specialist drugs needed eg. Hypertonic saline?
	5. COMMUNICATION
☐	Exact destination & Handover location confirmed?
☐	Team have phone with numbers of consultant, receiving unit & Money?
☐	Relatives briefed; information and directions given?
☐	Receiving hospital informed of ETA. ***Remember to update ETA approx 20 minutes pre-arrival.***